

Convalescent Snakes & Ladders: Phenomenological and Thematic Mapping

This thematic mapping supplements the papers:

van Rooyen, A., *A Missing Map: Exploring Hermeneutical Undersight and an Imaginary of Post-Acute Recovery through a Patient-Generated Educational Artefact* (under review).

and

van Rooyen, A., *Beyond Discharge: The Epistemic Work of Post-Acute Recovery* (under review).

Methodological Note:

This document groups the game's squares into a small number of recurring experiential domains to make visible patterns in post-acute recovery that are often encountered clinically but under-articulated in anticipatory guidance.

The clusters are intended as analytic lenses rather than diagnostic categories – they organise the squares by shared features of lived experience, not by symptom taxonomy or presumed mechanism. Individual squares may therefore appear in more than one cluster, reflecting the way recovery is experienced as overlapping and unstable rather than neatly segmented.

The purpose of this thematic mapping is pedagogical rather than exhaustive, offering clinicians and educators a way to see how ordinary post-discharge difficulties accumulate, intersect, and generate epistemic labour for patients in the absence of a shared recovery framework.

The clusters are:

1. Dysregulation and Instability
2. Energetic Instability and Capacity Mismatch
3. Misinterpretation, Anxiety, and Epistemic Vulnerability
4. Social Exposure, Performance, and Misrecognition
5. Everyday Logistics as a Site of Failure
6. Absurdity, Mundanity and Small Human Moments

Dysregulation and Instability

This cluster brings together squares that depict failures of bodily regulation across multiple systems, where ordinary sensory, positional, or physiological inputs produce disproportionate and often unpredictable effects. For patients, the experience is often one of bodily unreliability, that is signals cannot be trusted, thresholds shift without warning, and attempts at self-monitoring yield inconsistent or contradictory information. While these patterns are clinically familiar, they are rarely anticipated or explained at discharge.

Square	Text
9	You have to sit on the floor to stop the kitchen spinning while the kettle boils.
10	You are both ravenous and repulsed by most foods.
15	You shave your legs without falling over. Orthostatic what now? You celebrate ... prematurely.
16	You stand chatting in the park for 10 minutes and feel nauseous, sweaty, and faint - you beat a glacial retreat.
23	You find leftover hospital snacks in the fridge and have a full body shudder of revulsion.
24	Walk a quarter of a mile feeling dizzy the whole way. Will do it again tomorrow. Worst hobby EVER!
25	You're cold, you put on a jersey. You are now hotter than the Earth's molten core.
33	Walk to Greggs for fresh air, drop your steak bake and knock over your coffee. Fresh air is for losers.
41	You cry because you are cold and your jersey is out of reach.
43	You roll over in bed. Everything pops like bubble wrap.
44	Your arm hurts and you don't know why? It's your phantom PICC line.
47	You cry because you are awake + PAIN.
48	The sun is SO loud!
61	Your legs burn like fire ants have taken up residence in your epidermis.
66	You are dizzy standing up. You sit down. You are dizzy sitting down. You go to bed.
72	You turn on your desk light and feel like you're being interrogated by the Stasi.
80	You are an obnoxious baby in physio. you cry, you sleep, you are filled with regret + PAIN.
83	Your body temperature is now in Cryogenic Mode.
84	You cry because ... honestly ... you forget why.
90	Noise has become a physical assault. You cover your ears in pain.
92	Your new nickname is 'Butterfingers'.

Energetic Instability and Capacity Mismatch

This cluster of squares depict post-acute recovery as a problem of energetic unpredictability rather than simple fatigue. Across these squares, difficulty arises from a persistent mismatch between intention and capacity – plans are made in good faith, signals are misread, and effort is undertaken without a clear sense of its eventual cost. Feeling well becomes an unreliable indicator of readiness, and success may carry a penalty. For patients, this produces a form of epistemic instability where the body behaves as an unreliable narrator, undermining attempts at planning, pacing, or self-trust.

Square	Text
2	Being upright smacks of effort, you opt to be horizontal – a solid first move.
3	You think you'll have just one more day in bed before you start being productive - adorable!
11	You haven't slept properly yet. You worry you are failing at recovery.
18	You tweak the shoulder you strained in hospital. Holding a cup of tea is now a Herculean task.
20	Today we sleep. Existentially, you are a deflated marshmallow.
24	Walk a quarter of a mile feeling dizzy the whole way. Will do it again tomorrow. Worst hobby EVER!
26	Blow your hospital energy budget on an ontologically rejuvenating Caper, but still cry outside the pharmacy.
27	You stood up. That's your quota for today. Lie down.
34	You realise the hospital energy budget was a figment of your imagination.
40	Your brain is in cosmic inflation; your body hasn't been outside for 3 days.
50	You scroll for an hour before getting out of bed, then realise you're worn out and need a nap.
51	You book next week's gym sessions. Next Week You cancels all of them.
57	Used up a week's energy posing at a lecture & parking on the 8th floor. Hubris, pure hubris.
59	Sleep. Wake. Snack. Sleep. It is what it is – this is what they mean by Zen.
62	Do 2 minutes on the rowing machine and feel 25 again. Sleep for the rest of the day - in your gym clothes.
71	Too tired to shower. Too filthy not to.
75	You reluctantly acknowledge that 'early convalescence' is 'a thing' and you're doing it.
76	You mistake a long drive for a low-effort activity. Expect BIG PAIN for at least 7 days.
81	You think 'maybe I'll go out today'. The <i>Gods of Convalescence</i> laugh at your plans.
87	You felt great for two days in a row – have some crushing fatigue ... and PAIN.
88	Walking pace geriatric tortoise on sedatives.
96	You have a day with no pain. You think you are cured – you are not.

Misinterpretation, Anxiety, and Epistemic Vulnerability

This cluster collates squares that depict failures of sense-making during recovery. Anxiety, self-blame, and overthinking emerge not as psychological excesses, but as responses to epistemic uncertainty. Patients attempt to make sense of delayed feedback by moralising difficulty (“I am failing at recovery”), catastrophising ambiguous symptoms, or retrospectively reinterpreting actions whose costs were not foreseeable at the time. Together, these moments show how post-discharge recovery becomes an epistemically vulnerable state – meaning is not given in advance, but must be assembled after the fact, through accumulation, contradiction, and delayed recognition.

Square	Text
6	You think about showering, dressing, shopping, cooking - these remain bizarrely abstract concepts.
11	You haven't slept properly yet. You worry you are failing at recovery.
34	You realise the hospital energy budget was a figment of your imagination.
45	Thought precedes action. You think about tidying. Action TBD.
46	Calves feel swollen? Google compartment syndrome & ruminate on lower leg amputation.
54	You insist you can do something. You cannot. The only person surprised is YOU.
58	“We forgot to mention” : “Too late you had to find out through lived experience”
75	You reluctantly acknowledge that 'early convalescence' is 'a thing' and you're doing it.
76	You mistake a long drive for a low-effort activity. Expect BIG PAIN for at least 7 days.
77	You try to rate your pain, but it involves the Fibonacci sequence.
87	You felt great for two days in a row – have some crushing fatigue ... and PAIN.
91	Can solve for 'x', cannot put away laundry.
93	Build another hair hamster, and check for bald spots.
95	“That's perfectly normal” : “Now you tell me!”

Everyday Logistics as Sites of Failure

This cluster gathers moments where recovery is negotiated not through dramatic setbacks or clinical events, but through the quiet attrition of ordinary tasks. Recovery here is characterised by stalled intentions, abandoned attempts, and adaptive resignation. The work of convalescence unfolds through dropped objects, uneaten groceries, unread emails, and half-finished routines. These moments expose not incompetence, but a structural mismatch between assumed capacity and lived reality. Everyday tasks require sequencing, initiation, memory, physical reach, grip strength, temperature regulation, sustained attention, etc. When these capacities are compromised, failure accumulates in small, repetitive ways that generate frustration, shame, and exhaustion disproportionate to the task itself.

Square	Text
3	You think you'll have just one more day in bed before you start being productive – adorable!
5	A friend reminds you groceries can be delivered. You order them. You will not eat them.
7	Deeply resentful at having to sort your own medication.
8	You open the inbox you haven't dealt with for 3 months. Not today either!
13	You emerge from your duvet cocoon, furious at needing to fulfil your responsibilities.
17	Sulk because there is no one to bring you snacks.
21	You restart an interrupted work project. You last half an hour before going back to bed.
22	Feel bad. Eat. Feel better. Surprise!
29	You cry because your food delivery is delayed.
33	Walk to Greggs for fresh air, drop your steak bake and knock over your coffee. Fresh air is for losers.
35	You buy more groceries you are too tired to prepare and will throw out.
39	You drop something on the floor, decide you never really needed it, it lives there now.
41	You cry because you are cold and your jersey is out of reach.
53	Your empty pill box is a violation of your civil liberties.
60	Put clothes in the washing machine. They are now trapped in an infinity wash for 7 - 10 business days.
63	Accept that your usual 6am start time is now 10am.
64	You buy another bottle of Coke you can't open. A random passerby must do it for you.
67	More groceries? You indefatigable optimist! You put another bin liner in, because ... well ...
71	Too tired to shower. Too filthy not to.
74	NHS text reminders drive you bonkers even though you still need them.
78	Wear an actual outfit with accessories and perfume. Back in your pyjamas by 3pm, but you smell great.
79	Eat an 'extended family' sized bar of chocolate. You would have preferred vegetables.
98	An acquaintance wants to visit. It's easier to unfriend them than tidy the house.

Social Exposure, Performance, and Misrecognition

This cluster brings together squares that foreground how recovery is socially negotiated through conversation, help, decision-making, and expectations about normality, productivity, and gratitude. Patients are required to perform coherence, improvement, usefulness, or resilience in order to remain legible to friends, colleagues, and clinicians. Offers of help, reassurance, or encouragement may be well-intentioned, yet still impose moral framings or arrive too late to reduce uncertainty.

Square	Text
31	A friend asks how you're doing - you give them a blow-by-blow account - you can never speak to them again.
38	Someone makes a decision for you, not with you. Autonomy Override Alert activated.
42	Your bestie visits, stays in an hotel, cleans your house while you watch, and leaves early. You LOVE her!
49	You skip a work meeting because work is DUMB. New levels of self-advocacy and petulance unlock simultaneously.
52	Someone tries to tell your story for you. You correct the narrative. Authorial integrity retained.
69	'You deserve a nap'
70	You tell your 100th person 'I'm not very good at stairs at the moment'.
73	You try to be helpful. You are not. You have created more work.
95	“That's perfectly normal” : “Now you tell me!”
98	An acquaintance wants to visit. It's easier to unfriend them than tidy the house.

Absurdity, Mundanity, and Small Human Moments

These squares capture the texture of recovery as lived, rather than endured. They foreground humour, boredom, minor victories, and everyday human moments as integral ways patients experience and make sense of convalescence. Humour operates here as a coping strategy and a cognitive reframe; mundanity as the backdrop against which recovery unfolds; and small human moments as fragile but meaningful markers of progress. Taken together, these moments resist a clinical framing of recovery as either heroic struggle or passive rest. Instead, they show convalescence as a mode of ongoing personhood, where laughter, vanity, pettiness, pride, shame, and play continue alongside pain and fatigue.

Square	Text
4	You leave the hospital bubble and get a cold. You used to think this was what 'very poorly' meant. You know better now.
12	You feel nostalgia for your favourite nurses.
14	You get a haircut and feel less like a feral hospital goblin.
19	Go to the theatre, manage all the steps, feel absurdly proud of yourself.
26	Blow your hospital energy budget on an ontologically rejuvenating Caper, but still cry outside the pharmacy.
30	You journal your experiences – you call them 'The Shit Fit Chronicles'
32	Your NHS correspondence pile is bigger than your pension plan.
36	You coin the term 'pus ferrets' to describe your drains and laugh until you cry. Core workout completed.
37	The Deliveroo man doesn't know your name, but he knows your secret code.
55	You forget the real world is not hospital and go to Sainsbury's in your pyjamas.
56	Attend a lecture & walk up 8 flights of stairs in the carpark without the handrail. You little champion!
59	Sleep. Wake. Snack. Sleep. It is what it is – this is what they mean by Zen.
63	Accept that your usual 6am start time is now 10am.
64	You buy another bottle of Coke you can't open. A random passerby must do it for you.
65	Check credit card statement. You've spent a month's salary on Just Eat. Feel depressed, order a burger.
78	Wear an actual outfit with accessories and perfume. Back in your pyjamas by 3pm, but you smell great.
82	You try to salute a stupid driver and realise your middle finger is now in the target audience for Viagra.
85	You spontaneously load and unload the dishwasher in the same 24-hour period. Heroic.
86	You buy groceries, cook them, and clean up all in the same hour. You nap to celebrate.
89	You finally understand that convalescence is a pattern of stable chaos. Your bed is the attractor.
94	Your new love of steak bakes is a culinary violation of everything you stand for – you eat another one anyway.
97	You play the 'I nearly died' card during a vicious round of Uno, your brother plays the +5 card.
99	You compare weight loss wattles with a colleague on Ozempic. You're winning.